



HEALTH INSURANCE CLAIM FORM

APPROVED BY NATIONAL UNIFORM CLAIM COMMITTEE 02/12

GLOBAL LIBERTY INSURANCE
COMPANY OF NEW YORK
P.O. BOX 100
PLAINVIEW, NY 11803

1. medicare ☐ (medicare #) ☐ medicaid ☐ (medicaid #) ☐ tricare ☐ ID#/DoD# ☐ champva ☐ (Member ID#) ☐ group health plan (SSN or ID) ☐ feca blk lung (SSN) ☐ other ☒ (id)		1a. insured's i.d. number (for program in item 1) 119951	
2. PATIENT'S NAME (Last Name, First Name, Middle Initial) AKEEM EVANS		3. PATIENT'S BIRTH DATE MM 05 DD 21 YY 1981 SEX M ☒ F ☐	
5. PATIENT'S ADDRESS (No., Street) 740 Empire Blvd		6. PATIENT RELATIONSHIP TO INSURED Self ☐ Spouse ☐ Child ☐ Other ☐	
CITY Brooklyn		CITY	
STATE NY		STATE	
ZIP CODE 11213		ZIP CODE	
TELEPHONE (Include Area Code) (718) 778-6972		TELEPHONE (Include Area Code)	
9. OTHER INSURED'S NAME (Last Name, First Name, Middle Initial)		10. IS PATIENT'S CONDITION RELATED TO:	
a. OTHER INSURED'S POLICY OR GROUP NUMBER		a. EMPLOYMENT? (Current or Previous) ☐ YES ☒ NO	
b. RESERVED FOR NUCC USE		b. AUTO ACCIDENT? ☒ YES ☐ NO	
c. RESERVED FOR NUCC USE		c. OTHER ACCIDENT? ☐ YES ☒ NO	
d. INSURANCE PLAN NAME OR PROGRAM NAME		10d. CLAIM CODES (Designated by NUCC)	
12. PATIENT'S OR AUTHORIZED PERSON'S SIGNATURE I authorize the release of any medical or other information necessary to process this claim. I also request payment of government benefits either to myself or to the party who accepts assignment below. SIGNED SIGNATURE ON FILE DATE 11/01/2011		11. INSURED'S POLICY GROUP OR FECA NUMBER FHP0717368-1	
		a. INSURED'S DATE OF BIRTH MM DD YY M ☐ F ☐	
		b. OTHER CLAIM ID (Designated by NUCC)	
		c. INSURANCE PLAN NAME OR PROGRAM NAME GLOBAL LIBERTY INSURANCE COMPANY OF N	
		d. IS THERE ANOTHER HEALTH BENEFIT PLAN? ☐ YES ☒ NO If yes, return to and complete item 9a-d	
		13. INSURED'S OR AUTHORIZED PERSON'S SIGNATURE I authorize payment of medical benefits to the undersigned physician or supplier for services described below. SIGNED SIGNATURE ON FILE	
14. DATE OF CURRENT ILLNESS, INJURY, or PREGNANCY (LMP) MM 09 DD 02 YY 2011 QUAL		15. OTHER DATE QUAL MM DD YY	
17. NAME OF REFERRING PROVIDER OR OTHER SOURCE Azu Ajudua		16. DATES PATIENT UNABLE TO WORK IN CURRENT OCCUPATION FROM MM DD YY TO MM DD YY	
19. ADDITIONAL CLAIM INFORMATION (Designated by NUCC)		18. HOSPITALIZATION DATES RELATED TO CURRENT SERVICES FROM MM DD YY TO MM DD YY	
21. DIAGNOSIS OR NATURE OF ILLNESS OR INJURY (Relate Items 1, 2, 3 or 4 to Item 24E by Line) 1. 723.1 2. 722.0 3. 722.6 4. 724.2 5. 737.30 6. 722.93 7. 722.6 8. 719.46		20. OUTSIDE LAB? ☐ YES ☒ NO \$ CHARGES	
		22. RESUBMISSION CODE ORIGINAL REF. NO.	
		23. PRIOR AUTHORIZATION NUMBER	
24. A. DATE(S) OF SERVICE From MM DD YY To MM DD YY		B. PLACE OF SERVICE	
C. EMG		D. PROCEDURES, SERVICES OR SUPPLIES (Explain Unusual Circumstances) CPT/HCPCS MODIFIER	
E. DIAGNOSIS POINTER		F. \$ CHARGES	
G. DAYS OR UNITS		H. EPSDT Family Plan	
I. ID. QUAL.		J. RENDERING PROVIDER ID. #	
10 04 11 10 04 11 11		72141 1 2 3 4 5 6 7 8 \$879.73 1 N NPI 183333	
10 04 11 10 04 11 11		72148 1 2 3 4 5 6 7 8 \$912.00 1 N NPI 183333	
10 17 11 10 17 11 11		73721 1 2 3 4 5 6 7 8 \$878.67 1 N NPI 183333	
25. FEDERAL TAX I.D. NUMBER 26-1323136 SSN EIN ☐ ☒		26. PATIENT'S ACCOUNT NO. 83	
27. ACCEPT ASSIGNMENT? (For govt claims, see back) ☐ YES ☒ NO		28. TOTAL CHARGE \$2,670.40	
29. AMOUNT PAID \$		29. AMOUNT PAID \$2,670.40	
30. Rsvd for NUCC			
31. SIGNATURE OF PHYSICIAN OR SUPPLIER INCLUDING DEGREES OR CREDENTIALS (I certify that the statement on the reverse apply to this bill and are made a part thereof) WINTER W STEVEN W. M.D. SIGNED DATE 11/01/2011		32. SERVICE FACILITY LOCATION INFORMATION STELTON RADIOLOGY CORP 1849 Utica Avenue Brooklyn, NY 11234	
33. BILLING PROVIDER INFO & PHONE # STELTON RADIOLOGY CORP 1849 Utica Avenue Brooklyn, NY 11234 (718) 676-5595			
a 183333		b	
a 183333		b	



STEVEN W. WINTER, M.D. P.C.

1849 Utica Avenue Brooklyn, NY, 11234
Phone:(718) 676-5595 Fax:(718) 709-7499

PATIENT NAME: Akeem Evans

DATE OF SERVICE: 10/04/2011

REFERRING PHYSICIAN: Azu Ajudua

MRI Cervical spine:

Multi-planar, multi-echo images through the cervical spine were performed.

At the C5/6 level there is a 1 mm retrolisthesis with a left posterolateral disc herniation that extends into the left anterior recess and into the left neural foramen. There is a smaller right posterolateral component to the disc herniation that encroaches into the right anterior recess at this level. Anterior disc extension is present.

There is also inferior cerebellar tonsillar ectopia extending approximately 1 cm below the level of the foramen magnum representing an Arnold Chiari Type I malformation. There is narrowing of the foramen magnum. There is no evidence of a syrinx.

Otherwise, there is no evidence of discogenic disease, disc bulge or disc herniation. There is no compromise of the cervical cord, thecal sac or neural foramina. There is no bony destructive lesion or paravertebral soft tissue mass.

- 1. At the C5/6 level there is a 1 mm retrolisthesis with a left posterolateral disc herniation that extends into the left anterior recess and into the left neural foramen. There is a smaller right posterolateral component to the disc herniation that encroaches into the right anterior recess at this level. Anterior disc extension is present.**
- 2. The inferior cerebellar tonsillar ectopia extending approximately 1 cm below the level of the foramen magnum representing an Arnold Chiari Type I malformation. There is narrowing of the foramen magnum. There is no evidence of a syrinx.**

Thank you for the courtesy of this consultation.

Winter Steven W., M.D.

Diplomate American Board of Radiology Board Of Education 1234567890.

Diplomate American Board of Radiology.



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1849 Utica Avenue Brooklyn, NY, 11234
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PATIENT NAME: Akeem Evans

DATE OF SERVICE: 10/17/2011

REFERRING PHYSICIAN: Azu Ajudua

MRI Left Knee:

Multi-planar, multi-echo images through the left knee were performed.

The patient is post-operative with plate and screws anteriorly in the proximal tibia extending to the level of the tibial tubercle. There is some localized distortion of the images involving the proximal tibia.

There are also appears to be some post-operative related artifact at the level of the distal patellar tendon which should be assessed clinically. Inhomogeneous signal of the patellar tendon indicates tendinosis/tendinopathy. There is also quadriceps tendinosis/tendinopathy.

Otherwise the cruciate and collateral ligaments and both menisci show no other regions of signal alteration or morphological abnormality. The articular surfaces and bone marrow signal of the knee are otherwise without morphologic or signal alterations, and the remainder of the soft tissues surrounding the knee are otherwise intact.

- 1. There is no meniscal or ligament tear.**
- 2. Patellar and quadriceps tendinosis/tendinopathy.**
- 3. Post-operative changes in the proximal tibia.**

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PATIENT NAME: Akeem Evans

DATE OF SERVICE: 10/04/2011

REFERRING PHYSICIAN: Azu Ajudua

MRI Lumbar spine:

Multi-planar, multi-echo images through the lumbar spine were performed.

There is a left convexity of the lumbar curvature.

There are posterior disc bulges at L3/4 and L4/5 this impresses on the thecal sac at L3/4 and is abutting the thecal sac at L4/5. L4/5 demonstrates a 1 mm retrolisthesis.

The most inferior discernible disc is a small transitional L5/S1 disc which demonstrates posterior disc bulging into the epidural fat.

There are reactive end-plate changes at the anterior/superior margins of L3 and L4.

Otherwise, there is no evidence of discogenic disease, disc bulge, disc herniation or compromise of the thecal sac, lateral recesses or neural foramina. There is no conus medullaris lesion, fracture, bony destructive lesion or paravertebral soft tissue mass otherwise demonstrated.

- 1. There is a left convexity of the lumbar curvature.**
- 2. Posterior disc bulges at L3/4 and L4/5 this impresses on the thecal sac at L3/4 and is abutting the thecal sac at L4/5. L4/5 demonstrates a 1 mm retrolisthesis.**
- 3. The most inferior discernible disc is a small transitional L5/S1 disc which demonstrates posterior disc bulging into the epidural fat.**
- 4. There are reactive end-plate changes at the anterior/superior margins of L3 and L4.**

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